Mr. Balraj Verma

Age / Sex : 64 YRS / M

Referred by : INDRA KIDNEY CARE & SURGICAL CENTER

Reg. no. : 6988

6988

Registered on : 21/10/2026 10:59 AM

Collected on : 21/10/2026 10:00 AM

Received on : 21/10/2026 10:01 AM

Reported on : 21/10/2026 11:00 AM

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SEROLOGY & IMMUNOLOGY

VIRAL MARKER

TEST	VALUE	UNIT	REFERENCE
HIV (CARD TEST)			
HIV - 1	NEGATIVE		
HIV - 2	NEGATIVE		
HEPATITIS C VIRUS, HCV	WEAKLY POSITIVE		
HBSAG	NEGATIVE		

Advice *RECOMEND FOR HCV ELISA/ RNA FOR CONFIRMATION*

HbsAg is the first serologic marker appearing in the serum 6-16 weeks following hepatitis B viral infection. In typical HBV infection, HBsAg will be detected 2-4 weeks before the liver enzyme levels (ALT) become abnormal and 3-5 weeks before the patient develops jaundice. In acute cases HbsAg usually disappears 1-2 months after the onset of symptoms. Persistence of HbsAg for more than 6 months indicates development of either a chronic carrier state or chronic liver disease. The presence of HbsAg is frequently associated with infectivity. HbsAg when accompanied by Hepatitis Be antigen and/or hepatitis B viral DNA almost always indicates infectivity.

Anti HCV card test interpretation
Reactive test result indicates presence of Hepatitis C virus infection. Active infection to be confirmed by HCV RNA PCR test. It cannot differentiate between the stages of Hepatitis C viral infection nor used to monitor the efficacy of treatment. Non-Reactive test result indicates Hepatitis C virus infection is unlikely.

HIV Interpretation
A negative result implies that no Anti HIV-1 & HIV-2 antibodies have been detected in the sample by this method. This means that either the patient has not been exposed to HIV-1 or HIV-2 infection or the sample has been tested during the “WINDOW PHASE” (before the development of detectable levels of antibodies). A positive result suggests the possibilities of HIV-I and / or HIV-II infection. However these results must be verified by a confirmatory test (IFA / WESTERN BLOT I-II) before pronouncing the patient positive for HIV-1 and / or HIV-2 infection. All reactive samples should be confirmed by using HIV Western Blot/PCR.

BIOCHEMISTRY

KIDNEY FUNCTION TEST (KFT)

TEST		VALUE	UNIT	REFERENCE
RANDOM BLOOD SUGAR		130.5	mg/dl	70 - 140
SERUM UREA	**H**	**87.54**	**mg/dl**	**18 - 55**
SERUM CREATININE	**H**	**8.54**	**mg/dl**	**0.72 - 1.43**
SERUM PHOSPHORUS	**H**	**5.66**	**mg/dl**	**2.5 - 4.5**
SERUM URIC ACID		6.30	mg/dl	3.5 - 7.2
SERUM SODIUM	**L**	**132.5**	**mmol/L**	**136 - 146**
SERUM POTASSIUM		4.88	mmol/L	3.5 - 5.1
ICALCIUM		1.15	mmol/l	1.13 - 1.33
PH		7.445		

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia 2. Large amount of dietary meat 3. Active acromegaly and gigantism	1. Pregnancy 2. Increasing age (reduction in muscle mass)

~~~ End of report ~~~

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